

in person

S:

f/u

cervical LAN

review ultrasounds and blood work performed recently

Over the past four to five years, there has been a gradual enlargement of lymph nodes. Previous biopsies and blood tests have not provided conclusive results. There are no significant symptoms indicative of infection or cancer.

The patient has not reported any rashes or diarrhea and generally feels well, aside from the effects of jet lag from recent trip to China.

we review most recent imaging studies to monitor LAN progression and discuss overall perspective on the situation.

he feels fine but would like to have some degree of diagnostic certainty as to the underlying cause of the persistent LAN.

he has also had h/o hematuria and we review prostate, renal US as well as urine cytology

O:

PE:

well appearing

VSS WNL afebrile

CTA BL good air movement in all lung fields

NSR S1 S2 no murmurs no JVD no pitting edema

Abdomen non distended

normal gait

there is a notable enlargement of lymph nodes in the upper anterior neck.

These lymph nodes are neither tender nor rock hard upon palpation.

3/24/2025:

UA WNL UCx Negative

Urine Cytology Negative

ACR WNL

CRP 9.1 H

3/24/2025: US Neck, b/l Groin, Abdomen, Pelvis, b/l Axilla

Well defined hypoechoic masses again noted in the anterior neck with thick cortical mantles.

right level 2A 37.8x11.5x27.5 mm (larger)

right level 2A 23.3x5.8x19.9 (new)

left level 2A 22.3x9.4x19.6 mm (larger)

left level 2B 19.9x7.8x16.2 (larger)

Right side groin notable for well defined hypo and hyperechoic masses with thin cortical mantels with dominant ones as follows medial to neurovascular bundle 24x7.5x13.8 mm.

left side crying similar findings as right side with dominant ones medial to neurovascular bundle 15.3x4.2x12.8 mm

Normal bilateral renal ultrasound

normal prostate volume 13.2 CC post void 63 CC

In the region of the palpable lump on the right side axilla well defined hypo and hyperechoic masses in the subcutaneous tissue with thin cortical mantels dominant as follows 21.5x7.7x10.8 mm, 18.8x5.8x7.9 mm

In the region of the palpable lump on the left side axilla well defined hypo and hyperechoic masses in the subcutaneous tissue with thin cortical mantels dominant as follows 25.5x8.4x10.4 mm, 16.4x5.6x8.3 mm

1/2025:

CBC WNL HGB 162 PLTS 212 WBC 7.1 FERRITIN 201

UA HEMATURIA ACR WNL

A1C 5.4

CMP WNL CR 68 GFR > 120

T BILI WNL

UREA WNL

URATE WNL

TC 3.80 TG 0.76 HDL 1.13 LDL 2.32 NHDLC 2.67 (NO STATIN)

ESR WNL

CRP 5.0 (<5.0)

ANA NEGATIVE

ENA SCREEN NEGATIVE (SSA, SSB, RNP, SCL70, JO1)

RF NEGATIVE

Anti DS-DNA NEGATIVE

IgA WNL IgG WNL IgM WNL

SPEP WNL

VIT D 25 OH 56.6 L

B12 WNL

PTH WNL

TSH WNL FT4 WNL

Ca WNL

Stool culture negative, PCR negative, Blastocystis detected

EBV IgG Reactive

CMV IgG non reactive

Lyme Tier 1 EIA Non reactive

HAV IgG DETECTED - infection vs vaccination

HCV NEGATIVE

HIV Negative

9/23/2024: US Neck

moderate to markedly enlarged LN in upper anterior neck

right 2A 32x10x28 mm (larger)

right 2A 22x8x16 mm (stable)

left 2B 15x10x11 mm (new

atypical for both benign and neoplastic adenopathy, perhaps non infectious inflammatory process

consider FNA

12/1/2023:

UA WNL NO HEMATURIA NOTED

CR 68 GFR > 120

ACR WNL

LYTES WNL

12/1/2023: US Neck and Renal

moderately enlarged LN upper anterior neck w/ thick cortical mantles, unchanged or slightly larger. could still be reactive. neoplastic process less likely.

normal b/l kidney and normal bladder

12/23/2022: US NECK

interval decrease in size of benign appearing b/l upper anterior neck LN

likely reactive

12/23/2022: US b/l renal and bladder: normal

8/8/2022

CBC WNL HGB 164

UA WNL ACR WNL

A1C 5.0

CMP WNL GFR > 120

TC 4.87 TG 0.98 HDL 1.62 LDL 2.80 NHDLC 3.25 (NO STATIN)

TSH WNL

GC/C/T URINE NEGATIVE

2/2022 CT ABDOMEN PELVIS W/ IV CONTRAST

tiny cortical densities likely cysts noted on kidneys

a few prominent ileocolic LN up to 7 mm - stable - nonspecific

2/2022 CT CHEST w/ IV contrast

no intrathoracic LAN

multiple small b/l lung nodules largest RUL 4 mm middle lobe 3 mm RLL sub-pleural 4 mm LLL 3 mm. emphysematous background

repeat study in 6 months

1/2022: EGD / CSP from Thornhill Endoscopy Center

antrum mild chronic gastritis. ileal mucosa w/ mild chronic active ileitis

no malignancy. no h. pylori

12/2021 MRI HEAD AND NECK W/ AND W/O CONTRAST

scattered benign appearing LN w/i both sides of neck including parotid glands these do not appear suspicious - largest 18 mm right side jugulodigastric LN

no suspicious lymph nodes throughout head and neck

12/2021: LDH WNL

11/2021:

ANA 1:80 (considered ULN) speckled pattern homogenous pattern

ESR WNL CRP WNL

SM NEG CK NEG ds DNA NEG RF NEG CCP NEG ENA NEG

11/2021: 2/2 PPD neg

11/2021: Cystoscopy by Dr. Sidney Radomski @ TWH

normal urine cytology pending needs annual US a/p

10/2021: CT a/p - hematuria protocol: normal

10/2021: US LN FNA left Neck Zone 2

negative for malignancy - reactive lymphoid population - negative pap stain

T cell CD4/8 ratio 3.13:1

small B cell population negative for aberrant antigen expression

10/2021: US neck

dominant LN left upper anterior neck w/ moderately thickened cortical mantle

ddx includes neoplasia in appropriate context

8/2021: KUB and renal US: ?distal left ureteral calculus?

8/2021:

UA WNL - urine cytology negative for high grade urothelial carcinoma

8/2021: UA hematuria UCx negative

7/2021:

CBC WNL

UA hematuria ACR WNL

A1C 4.9

CMP WNL

TC 3.99 TG 1.20 HDL 1.42 LDL 2.02 NHDLC 2.57

Testosterone WNL Free Testosterone WNL

TSH WNL

GC/C negative HAV immune HBV neg HCV neg HIV neg VDRL neg

A/P:

Discussed Today

persistent and in fact enlarging asymptomatic neck supra-clavicular LAN of unclear cause. associated also with ileocolic LAN, scattered pulmonary nodules and recurrent episodes of microscopic painless hematuria.

setting of non smoker. no B symptom, feels very well.

investigations:

10/2021: US neck: dominant LN left upper anterior neck

10/2021: US LN FNA left Neck Zone 2

negative for malignancy - reactive lymphoid population - negative pap stain

T cell CD4/8 ratio 3.13:1. small B cell negative for aberrant antigen expression

11/2021:

ANA 1:80 (considered ULN) speckled pattern homogenous pattern

ESR WNL CRP WNL SM NEG CK NEG ds DNA NEG RF NEG CCP NEG ENA NEG

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scattered benign appearing LN b/l neck including parotid glands do not appear suspicious - largest 18 mm right side jugulodigastric LN

no suspicious lymph nodes throughout head and neck

1/2022: EGD / CSP from Thornhill Endoscopy Center

antrum mild chronic gastritis. ileal mucosa w/ mild chronic active ileitis

no malignancy. no h. pylori

2/2022 CT ABDOMEN PELVIS W/ IV CONTRAST

a few prominent ileocolic LN up to 7 mm - stable - nonspecific

2/2022 CT CHEST w/ IV contrast

multiple small b/l lung nodules largest RUL 4 mm middle lobe 3 mm RLL sub-pleural 4 mm LLL 3 mm on emphysematous background

6/2022: Pulmonologist Dr. Harvey Hoi-Wai Wong evaluated and noted negative negative sputum culture and AFB negative - TB was considered unlikely. The small nodules on CT were considered unlikely to be significant and repeat imaging was considered optional and declined repeat.

6/2022: normal laryngoscopy w/ ENT Dr. Alexander Osborn

12/23/2022: US NECK

interval decrease in size of benign appearing b/l upper anterior neck LN

likely reactive

12/1/2023: US Neck and Renal

moderately enlarged LN upper anterior neck w/ thick cortical mantles, unchanged or slightly larger. could still be reactive. neoplastic process less likely.

9/23/2024: US Neck

moderate to markedly enlarged LN in upper anterior neck

atypical for both benign and neoplastic adenopathy,

perhaps non infectious inflammatory process. consider FNA

3/24/2025:

UA WNL UCx Negative Urine Cytology Negative ACR WNL

CRP 9.1 H

3/24/2025: US Neck, b/l Groin, Abdomen, Pelvis, b/l Axilla

Well defined hypoechoic masses again noted in the anterior neck with thick cortical mantles, enlarging. max 37.8 mm

b/l groin notable for well defined hypo and hyperechoic masses with thin corticle mantels upt to 24 mm.

Normal renal ultrasound. Normal prostate. b/l axillary LAN

1/2025:

CBC WNL

UA HEMATURIA ACR WNL

ESR WNL CRP 5.0 (<5.0)

ANA NEGATIVE ENA SCREEN NEGATIVE (SSA, SSB, RNP, SCL70, JO1)

RF NEGATIVE Anti DS-DNA NEGATIVE

IgA WNL IgG WNL IgM WNL SPEP WNL

Stool culture negative (Blastocystis detected)

EBV IgG Reactive CMV IgG non reactive Lyme Tier 1 EIA Non reactive

HAV IgG DETECTED - infection vs vaccination HCV NEGATIVE

HIV Negative

a/p

unclear what is causing entirely asymptomatic, persistent (enlarging) cervical LAN, recurrent microscopic hematuria w/ reassuring cytology and cystoscopy.

for second opinion we referred him to IM Dr. Stephen Glazer and whose input and additional investigations are appreciated.

we discussed the possibility of total body gallium scan which was a suggestion, for now would like to avoid further radiation which is reasonable particularly as he is asymptomatic.

with respect to HAV IgG, EBV IgG and Blastocystis findings he has no symptoms to suggest chronic EBV, normal LFTs suggest against acute HAV infection as well he was noted to be HAV immune in 2021

he has no active enteric symptoms as well Blastocystis is considered usually a non pathogen.

the mild active ileitis noted in CSP in 2021 is similarly asymptomatic as he has no GI complaints and is not losing weight.

We discussed the unlikely possibility of a previously false negative LN Bx and he would be willing to entertain a repeat Bx - I will refer him back to ENT for evaluation for repeat Bx.

It is noted that his CRP has been trending upwards and we will repeat this along with CBC and peripheral cell smear. LDH, SPEP and prior CBCs have been reassuring.

I asked him to provide me a comprehensive travel history which he did email to me after the appointment. as listed below. we can consider this travel history as well from an infectious disease perspective in future.

USA

Mexico

Dominican Republic

UK

Germany

France

Portugal

Spain

Belgium

Italy

Russia

Finland

Morocco
Netherlands
France
Monaco
Czech Republic
Croatia
Switzerland
Romania
Austria
Cuba
Japan
China

He is scheduled to follow up with Dr. Glaser in mid-May.

we discussed this could represent an as yet undx rheumatologic or autoimmune process vs a subclinical infection and I think it would be reasonable for us to consult infectious disease for their opinion.

I will place a referral to infectious disease at this point.

additionally we did discuss that advancements in AI / LLM technology has allowed identification of rare or difficult to dx diseases and he provided me verbal consent to query an AI model with his story and de-identified partial medical records to see if we can learn something from this.

hematuria NYD in setting of never smoker

investigations:

11/2021: Cystoscopy with Dr. Sidney Radomski @ TWH

normal urine. needs annual US a/p

12/2022: bladder and renal sono - WNL

12/1/2023: UA WNL CR 68 GFR > 120 ACR WNL LYTES WNL

12/1/2023: US Renal: normal b/l kidney and normal bladder

1/2025: microscopic hematuria

3/24/2025: UA WNL UCx Negative Urine Cytology Negative ACR WNL

3/24/2025: US Abdomen, Pelvis

Normal bilateral renal ultrasound. normal prostate volume 13.2 CC post void 63 CC

a/p

most recent urine studies and renal US WNL

continue annual sonography - repeat in 3/2026

not discussed today

hemorrhoids

based on description sounds more likely external

1/2022: EGD / CSP from Thornhill Endoscopy Center

antrum mild chronic gastritis. ileal mucosa w/ mild chronic active ileitis

no malignancy. no h. pylori

a/p

anuso HC

referred back for evaluation of RBL vs hemorrhoidectomy

referred to CR surgery as it sounds like he may need hemorrhoidectomy

acne, very mild

trial rejuva

incidental finding of active ileitis on CSP

in setting of familial history of ulcerative colitis

no other symptoms consistent w/ IBD-CD such as persistent diarrhea, abdominal pain, mucous in stool

defer further evaluation

monitor for any new symptoms

